

MediSync Demo Script

Doctor and Administrator Portals

Total running time 6:00 • Doctor 2:20 • Administrator 3:40

Before you start. Sign in at maps-system-three.vercel.app/login. Have two browser tabs open and already logged in — one on the doctor portal, one on the administrator portal — so no demo time is spent typing passwords. Doctor: skim@medisync.health / doctor123. Administrator: admin@medisync.health / admin123. Set the browser to roughly 1600px wide so the desktop layout with the navy sidebar is showing.

Pace. Every spoken line is written to be read at a normal speaking pace. Read aloud it runs about 4:20, and the timestamps allow roughly five seconds a beat for clicking, which lands the whole thing near 5:20 — leaving some room to slow down. If you are running long, see the note at the end for the two beats that can be dropped.

Part 1 — The Doctor Portal 2:20

0:00 Land on the doctor's schedule

Do: Start on the doctor tab, already signed in as Dr. Sarah Kim. The day view is the landing page.

Say: This is what a physician sees when they sign in — their own day, nothing else. Each row is one patient, the reason they are coming in, and where the visit is being held. A physician does not manage the practice; they work their list, so this is the whole of their front page.

0:25 Switch Day → Week → Month

Do: Click the three view buttons in turn. Land back on Day.

Say: The same book at three zoom levels. The week and the month are colour-coded by status, so a provider can see at a glance where the gaps are and which bookings are still waiting on the front desk to approve them.

0:50 Complete a visit and record a note

Do: Click **Complete visit** on a confirmed morning appointment. Type a short note. Save.

Say: After the patient has been seen, the physician closes the visit out and writes the note against it. This is the one action in the system that only a doctor can take — an administrator can see that a slot came and went, but they cannot state that a consultation happened, so the system refuses it from them and names the physician who owns it.

1:25 Open a patient chart

Do: Go to **My Patients**, click a patient, scroll to the medications block. Open **Prescribe**, then close without saving.

Say: Every patient this physician has actually seen, and their chart — past visits with the notes

recorded at each, and the medications written for them. The physician can prescribe from here, or discontinue something they wrote.

1:50 Approve a refill request

*Do: Go to **Refill Requests**. Approve one. Point at the toast confirmation.*

Say: And this is the physician's own approval queue. A patient asks for more of a prescription, it lands here, and the decision is theirs. Approving notifies the patient straight away — which brings us to the other side of the practice.

Part 2 — The Administrator Portal 3:40

2:20 Clinic overview

*Do: Switch to the administrator tab. Land on **Overview**.*

Say: The clinic administrator gets the opposite view — the whole practice at once. Total appointments, the no-show rate, the cancellation rate, and how many requests are sitting unapproved. Underneath, volume by specialty and how the work is spread across the physicians.

2:45 Work the approval queue

*Do: Click the pending count, or go to **Appointments**. Approve the first request. Watch the queue shorten.*

Say: This is the job this screen exists for. A booking arrives as a request, not an appointment, and it stays pending until clinic staff accept it. Approving it confirms the patient's time and notifies them. The administrator can also move it or turn it down — but every one of those needs a reason, because a cancellation with no reason recorded is a row the practice can never learn anything from.

3:20 Switch the book to the calendar

*Do: Click the **Calendar** tab above the filters. Click a busy day to open it.*

Say: The same set of appointments as a month. Colour is status, so an administrator can see the shape of the fortnight without reading a single row, and clicking any day opens that day's full book.

3:45 Open a patient from the queue

Do: Click a patient's name in the table. The patient profile opens.

Say: Clicking a patient's name opens who they are — contact details, insurance, and every visit they have on the book. Notice what is not here: no clinical notes, no prescriptions. Administrators schedule care, and scheduling does not require reading somebody's chart, so the system does not send that data to this screen at all.

4:15 Physician schedules and a block (the important one)

*Do: Go to **Schedules**. Type two or three letters into the physician search. Pick someone. Show the weekly windows, then the **Calendar** tab.*

Say: Availability is a weekly pattern, not a list of slots — a window with a start, an end, a slot length, and the site it is held at. The calendar shows what that pattern actually produces on real dates, and how many bookable slots each day yields.

4:50 Block a date range

Do: Click **Add block**, pick a range that already has bookings, save. Point at the number in the banner.

Say: Now block the physician out for a week's leave. Those days disappear from patient search immediately — but look at what it tells us: the appointments already booked inside that week are not cancelled. The patients keep their place, and every one of them is flagged for rescheduling and told. Silently deleting them would be the easy thing to build and the wrong thing to do.

5:25 Finish on the reports

Do: Go to **Reports**, open **Provider Utilization**, then point at **Download CSV**.

Say: And all of it feeds the reporting. Provider utilization measures the slots that were taken against the slots the schedule actually offered — expanded from those same weekly windows, with the blocked days removed, so a physician on leave is never measured against time they never had. Every report exports to CSV exactly as shown. That is MediSync.

If you are running behind. Drop the **0:25** view-switching beat and the **3:20** calendar beat — the patient portal will already have shown a calendar, and neither is load-bearing for the story. That buys roughly fifty seconds. Do not cut the block at **4:50**: it is the one moment that shows the system making a decision rather than storing a form, and it is what the utilization figure at the end depends on.

Questions you should expect, and the short answers.

Why can't an administrator complete a visit? Because completion is a clinical assertion, and it is the record every workload and utilization figure is built on. The front desk can see that a slot passed; only the provider knows whether the patient was seen.

What stops two patients booking the same slot? A database constraint, not a check in the code. Two people booking at the same instant would both pass a read-then-write check; the second write here simply fails, and the patient is asked to pick another time.

Are the email and text reminders real? The channels, the per-user preferences and the message wording are all built and fire on every appointment event. The final hand-off to a carrier is simulated, because choosing a messaging vendor is a decision a clinic makes at deployment. A reminder sent on a schedule ahead of a visit is still future work.